Glocal Care: Can the Trust Behind Tulip Med Cross a Border?

A passion project in global marketing by Ibragim Sultan

“She will tell you what you don’t need.” This is how Karlygash Temirbolat, the founder of Tulip Med, sums up the reputation she has built in Astana (Temirbolat, 2026). I heard the sentence, which I have translated from Russian, during a ten-minute face-to-face interview that I conducted with her on 5 August 2026 for this project, and it stayed with me because it captures how she advises patients.

At Tulip Med, I kept coming back to one question: how do you market an aesthetic treatment when the most honest thing a clinic can say is that it might be wrong for the person asking? I began to notice how a few words in an Instagram post could shape what someone expects from a treatment and whether they feel comfortable asking questions. I explored that question in this project, using audience research to explore how Tulip Med could make its services appealing while being candid about what patients can expect.

The clinic’s reputation developed over two decades. Temirbolat began practising as a doctor in Astana in 2000, and from 2005 her clinic grew into a brand with more than twenty years on the market. She told me that word of mouth contributed to that growth, that Instagram is her main channel, and that she uses educational content to explain procedures and recovery. The clinic also works with the Korean brands Soskin and T-med, so it already sits inside a global supply chain even though its patients are mostly local.

This raises the question behind my project. A reputation of this kind is built face to face, over years, in one city. Can it travel? I ask whether Tulip Med can attract patients from Russia to Astana, and I argue that the answer depends on one idea from international marketing: **glocalisation**. The clinic should keep its promise global, meaning the same honest approach to clinical judgment wherever the patient lives, and make everything around the promise local, meaning language, price, travel, channels and follow-up. I propose a small pilot in inbound medical tourism rather than a branch in Russia, because a pilot tests the promise before the clinic spends money on a presence abroad.

This project is personal to me because Tulip Med was where I first saw myself pursuing a career in marketing. When the clinic invited students to volunteer alongside its social media specialist, I applied, interviewed, and was selected. As I got to know Tulip Med through its

Instagram, I began paying attention to how a well-known clinic in Astana communicated what made it distinctive and lived up to the reputation it had built. I wanted to know what prospective patients actually took away from those posts.

A flat world with very different patients

Globalisation connects information, services, capital and people across borders. Friedman (2005) popularised the image of a world where the playing field is being levelled. For a clinic, the change is visible online: a person in Russia can find a clinic in Kazakhstan online, read reviews, compare prices and decide whether a trip is worth it. The same connection works in the other direction, because Tulip Med's message now competes with every other clinic that patient can find.

Companies seeking long-term growth should consider expanding internationally to reach new customers, strengthen brand recognition, and increase sales. Levitt (1983) argued that selling a common offering across countries allows firms to benefit from economies of scale. However, customer expectations vary across markets, and a marketing strategy that works at home may need to be adapted to appeal to local customers (Douglas & Wind, 1987).

This approach is known as glocalisation. A company retains its core brand identity while adapting its marketing messages, channels, or offers to local preferences. When these changes attract enough customers to outweigh the additional costs of adaptation, they can improve profitability (Shoham, 1996).

The question for Tulip Med is not "global or local?" but "which element of the offer should be global and which local?"

What a burger chain can teach a clinic

I first looked at the fast-food chains KFC and Domino's, because both adapt their marketing across countries. Both keep a recognisable core. KFC keeps its fried chicken, its logo and its packaging standards, and Domino's keeps its pizza, its boxes and its standardised website and app. Both also adapt around that core. Both adjust prices to local costs, and Domino's raised menu prices in Germany after a currency change (Domino's Pizza, 2014).

KFC's response to a food safety crisis offers another example. In 2012, Chinese state television accused KFC of using growth hormones and antibiotics in its chickens. KFC

protected its reputation by building a platform for the Chinese market where it could answer complaints and questions about food safety (Sivertseva & Gao, 2014).

The comparison has limits. A burger can be tasted, judged and forgotten in half an hour. Medical care is different because the patient usually cannot judge it alone and often needs it to continue after returning home. For Tulip Med the “core product” is not a recipe. It is a way of deciding what a patient needs. That core can be standardised and carried abroad, but it carries a condition: the clinic must deliver it consistently, because Vrontis and Thrassou (2007) remind us that a firm’s country of origin also shapes how its offer is received. For a Kazakh clinic speaking to Russian patients, origin could be an asset, a doubt, or simply irrelevant. I treat it as a question for the pilot.

My working rule for Tulip Med is therefore simple:

- **Global (standardise):** the promise of honest clinical judgment, the standards of qualification, the clear explanation of options, and the refusal to promise individual results.
- **Local (adapt):** the language, the way price is shown, the travel plan, the channels through which people hear about the clinic, and the follow-up arrangements after a patient goes home.

A service gap appears when an element that cannot be delivered consistently across borders is promoted to a bigger audience. In that case, advertising can expose more patients to the same service problem.

Why a pilot in Astana and not a branch in Russia

Kazakhstan and Russia are both members of the Eurasian Economic Union. As I understand it, Article 28 of the EAEU Treaty concerns the internal market and customs duties in mutual trade in goods, while trade in services is dealt with separately in Section XV and Annex 16 (Treaty on the Eurasian Economic Union, 2014). The shared framework is useful context, but the tariff treatment of goods cannot show what is required to open a medical practice or promote healthcare services in another country. I therefore treat membership as background and build the proposal on patients who travel to the existing clinic in Astana. Any live pilot would first need a review of the rules covering communications, patient information and cross-border arrangements.

Johnston et al. (2010) define medical tourism through intentional travel abroad for non-emergency care. That definition lets me separate my target group from visitors who happen to use a clinic during an unrelated trip. My target is adults in Russia who would consider travelling to Astana specifically for a planned consultation or a clinician-approved service. A branch in Russia would answer a different question, which is whether the clinic can deliver care locally there. An inbound pilot tests whether the clinic and its founder's reputation offer enough value to justify the journey, and it gives only limited evidence about demand for a branch.

A founder as a glocal brand

Peters (1997) presents personal branding as an active way of shaping professional reputation, and Rampersad (2009) places professional vision at the centre of the process. Keller (1993) explains why this matters, because the associations people attach to a name become part of its value. The reputation of a person and that of an organisation can reinforce each other when the audience sees a credible link. The link runs both ways. Temirbolat's name may shape how a patient views the clinic, and a patient's experience with another clinician at Tulip Med may strengthen or weaken what her name means. The Korean brands the clinic works with add a third name to this relationship.

The critics of personal branding raise questions about authenticity. Shepherd (2005) notes that a personal brand is not the person but the outside perception of the person's presented abilities. Lair, Sullivan and Cheney (2005) raise ethical and authenticity questions about recasting the professional self as a brand. In my case this connects to something Temirbolat told me: her reputation rests on being willing to say what a patient does not need. She said that such a refusal may cost one visit but brings dozens of referrals. For this reason her voice should show real decisions, such as when a procedure can wait, rather than a polished slogan.

The strongest risk is dependence on one person. If foreign patients assume every visit will be with the founder and she is not available, the promise breaks. The clinic would have to say clearly who will treat each patient and show how the team applies the same way of assessing and advising. A distinctive cue helps people recognise a brand, as Romaniuk, Sharp and Ehrenberg (2007) showed, but an audience can remember the founder and still choose a provider closer to home. For this reason I would measure recognition, credibility and willingness to book separately.

Hanefeld et al. (2015) found that networks influence where medical tourists travel, and Crooks et al. (2010) describe the patient experience as a sequence of stages before, during and after travel. Together they suggest that a founder's voice may create a first contact, but referrals and checkable information will decide whether people act on it.

Who might travel: segments and a SWOT

Shared language gives Tulip Med access to several groups with different reasons to travel. I identify three as hypotheses for research. The first is adults who already have family or professional links with Astana, for whom a connection to the destination may reduce uncertainty, in line with the role of networks described by Hanefeld et al. (2015). The second is patients who actively compare clinics across borders for one service and may need stronger proof of expertise. The third is people attracted by a low advertised price, for whom the total cost of travel can quickly cancel the saving. I have no data on the size of any of these groups, so I begin with the first.

Keller (1993) also helps separate basic expectations from points of difference. Qualifications, clear prices and a credible follow-up plan are the minimum. The point of difference I propose is the founder's approach to judgment, shown by the clinic's readiness to explain when a procedure should be postponed or declined. The tension is that emphasising the founder too much may suggest a personal appointment that cannot be guaranteed.

Weihrich (1982) paired internal strengths and weaknesses with external opportunities and threats to derive strategic choices. Applied to the pilot, the analysis gives me the following picture.

- **Strengths.** Growth through recommendations and a library of educational content give the clinic an established voice and material to adapt (Temirbolat, 2026).
- **Weaknesses.** The offer may depend on the founder's visibility, and I have no basis for estimating Russian demand or the staff time international coordination would take.
- **Opportunities.** Russian-language educational content can reach interested adults before they travel, and people with links to Astana may face fewer barriers.
- **Threats.** Local clinics are closer and easier to reach for follow-up. Transport costs and exchange rates can change the total price, and a breakdown in coordination after the patient goes home could damage trust in both the founder and the clinic.

Strength combined with opportunity supports a founder-led educational campaign aimed at one origin market. Weakness combined with threat argues for starting with services the team can assess and follow up reliably. If bookings happen only when the founder is free, more traffic will lengthen waiting times before it improves the economics. I would therefore compare the contribution per completed visit, after acquisition, coordination and follow-up costs, together with measures of patient experience.

Designing the glocal journey

I use the marketing mix, to identify what the clinic can standardise and what it needs to adapt, and I follow the patient's journey from first enquiry to follow-up.

Product. The journey starts with a clear difference between an initial enquiry, a clinical assessment and any procedure offered afterwards. Before booking travel, a patient should understand the limits of any preliminary advice and what happens if the in-person assessment changes the plan. Clinicians, not marketers, should choose the services for the pilot, with attention to follow-up needs and travel feasibility.

Price. Price should be shown in tenge with an indicative rouble equivalent that carries a date, so that exchange-rate movements are visible. The Domino's case in Germany reminds us that exchange-rate changes can affect local prices. The fair comparison is the total cost and effort of the trip against a credible local option, because the clinic's own price alone cannot prove a cost advantage.

Place. Distribution begins online and ends in Astana. A dedicated Russian-language page can explain the appointment steps, the expected time in the city and the way to contact the clinic before and after travel. The visit and the follow-up plan should be designed together, including what a patient should do if they need local care after returning home. This is where the glocal idea matters most, since access depends on both online booking and travel to Astana.

The patient also takes part in the service. Chan, Yim and Lam (2010) studied 349 pairs of customers and employees at a financial institution in Hong Kong and the United States. They found that customer participation can create value, but that the effect depends on the cultural values of both the customer and the employee. Their setting is financial and not medical, and they did not compare Russia and Kazakhstan, so I use the study only as a frame. It suggests two questions for the pilot: what a patient from abroad is asked to do before and after the visit,

and which physician suits which patient. My interviews point to the same area, because people asked for a written summary, a named contact and a clear plan for a check-up at home.

Promotion. The choice of channels depends on what patients in Russia can access. Instagram is the clinic's main channel, yet access to it in Russia has been restricted since 2022, and some reports in 2026 describe growing restrictions on Telegram as well. Some Russian users may therefore reach such platforms only through workarounds, or turn to domestic platforms. Five people in Russia answered my survey after I reached them on Instagram, so the channel still works for some people, although five answers cannot show how many. KFC's response in China suggests the practical lesson, which is to build a channel where the audience really is. I would therefore treat the clinic's channel mix, and the current rules on promoting medical services in Russia, as a first item to verify. I would also test two versions of the Russian-language page: one that leads with Temirbolat explaining how she approaches decisions, and one that leads with the care team and the care pathway. Both would give equivalent information on services and costs, and the traffic would be comparable where possible. All communications should describe expected processes and avoid promises about individual results.

What two physicians and five Russian adults told me

Besides the founder interview, I collected two further pieces of evidence. On 3 August 2026 I interviewed, face to face, two physicians who had worked at clinics in Yekaterinburg and Novosibirsk before joining Tulip Med. I asked 15 questions in six blocks, from their background to what the clinic should avoid (Physician 1 & Physician 2, 2026). From 3 to 10 August I also invited 30 Instagram users at random to answer six open questions about travelling to Astana for aesthetic medicine. Five completed it, a response rate of 17%. They are aged 25 to 52 and live in Omsk, Yekaterinburg, Novosibirsk, Tyumen and Chelyabinsk, and none of them is a patient of Tulip Med (Sultan, 2026). I translated their answers from Russian myself, and I refer to them by first name, age and city. The two physicians and Karlygash Temirbolat agreed to the use of their answers, and to being mentioned. The full answers are in Appendices A and B.

The main finding surprised me, because it is not about price. Both physicians said the question they heard most often was whether a second trip would be needed. The survey points the same way. Two of the five would travel, two might, and one would not. Four of the five would consider a single visit, but only if no repeat trip is needed or if they are told in advance.

The main barrier was named once each: trust, time, follow-up, price and travel logistics. For a patient, a second trip is the point where the global promise meets the local reality. Physician 2 added that uncertainty about timing could matter more than the consultation fee.

> *“If a repeat in-person visit is needed, it is important for me to know in advance.” (Anna, 29, Omsk)*

Three more results bear directly on the ideas in this essay. First, the founder’s name works as an entrance and not as the deciding factor. One respondent said it matters that the founder herself sees her. Two said her name attracts attention but they choose by specialist, and two said it did not matter. Physician 1 said her name often gives patients their first introduction to the clinic, and advised against advertising that could leave people expecting an appointment with her personally. This is the dependence risk I described earlier, now seen from inside the clinic. Second, four of the five wanted to know exactly who would see them, and others asked what the price includes, what may cost extra, how cancellation works and whether they would receive a written summary. That is the clear explanation of options which I placed in the global layer, and the people I asked wanted it before booking. Third, the respondents who were open to travel linked it to something else: visiting relatives in Astana or working there. This supports the first segment I proposed, adults with existing links to the city. Respondents also said they would first hear about a clinic through recommendations, social media or search, then check reviews of the physician, and often book by messenger.

These answers help define what the pilot would test. I would begin with second-opinion consultations for people from Russia, as Physician 1 suggested. The pilot would follow five steps. The founder explains her approach on social media, and staff ask each person where they first heard of the clinic. Staff check before booking whether one visit is enough and whether the patient can have an in-person check at home. One page states who will see the patient, what the price includes, possible extra charges, cancellation and replacement terms, and that a consultation may end with advice to wait. The clinic introduces its physicians early. After the visit the patient receives a written summary, a named contact and an agreed check-in date, and the summary says which questions can be handled remotely and when an in-person assessment is needed. The stop rule is simple: if interested patients still find the trip too complicated, the pilot does not expand. Physician 2 made the same point, and also warned against judging a campaign by the number of messages alone.

How the pilot would work and what it cannot show

The first step is to choose one Russian city by comparing transport access, local alternatives and observable search interest. Brief interviews with adults who might travel would show why they would go or refuse and what information would change their mind. The interviews and survey reported above are a first version of this step, and a live pilot would repeat them with more people in the chosen city. A small campaign would then follow four steps: the share of visitors who send a relevant enquiry, the share of enquiries that become scheduled assessments, the share of booked patients who arrive, and the share of patients needing follow-up who complete it. These figures can be set against acquisition costs and staff time. Many enquiries with few arrivals would point to travel and scheduling barriers. High engagement with the founder but few relevant enquiries would suggest that interest in the founder is generating few enquiries.

A related question comes from my own work. As an intern working with the SMM manager in the marketing team at Tulip Med, I proposed an idea for a student discount, and the clinic put it into practice. In total, 38 students signed up. My interviews suggest that price is not the main barrier for patients from Russia, and Physician 2 warned against discounts that push people to buy a ticket before they understand the terms of the visit. A next step would therefore be to test whether a discount changes willingness to book, and what doubts a patient still has at a lower price. One possible doubt is quality: in cosmetology and medicine a low price may make a patient wonder whether cheaper products or drugs are used. None of my respondents said this, so it is a hypothesis to test and not a finding. What surprised me most during this project is that people are sometimes not ready to spend money on really good medicine, although health should come first. This is my own impression and not a result of the survey.

The project has a small evidence base. My evidence consists of existing literature, one founder interview, interviews with two physicians and answers from five Russian adults. The founder and the physicians all work at Tulip Med and have an interest in its success. Only 5 of the 30 people I invited answered, and people who reply to a survey invitation may be more interested in the topic than those who ignore it. They were reached on Instagram, where access in Russia has been restricted since 2022, so they may be more used to the platform than other people in their cities. Five answers cannot show how widely any view is shared, and I collected no cost data for flights or accommodation. Larger independent research would be needed to learn how common these perceptions are in Kazakhstan or Russia. I make no

estimate of market size, conversion or affordability, and results from one city would reflect that city's travel options and alternatives, so expanding to another would require testing again. Glocalisation also has a cost: Kotler et al. (2009) warn that adapting to each market takes time and money, and a small clinic would have to choose carefully where adaptation pays for itself.

Conclusion

I started this project with one sentence about telling patients what they do not need. Globalisation makes that sentence easy to send across a border, but it also puts it next to every competitor's message. Glocalisation gives Tulip Med a practical approach: keep the promise the same everywhere and adapt everything around it, such as the language, the price and the follow-up, to each place the patient comes from. A pilot that ties founder-led communication to arrivals and continuity of care would show where this approach works and what has to change before the clinic goes further.

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Appendix A. Interviews with two Tulip Med physicians

Face to face, 3 August 2026.

1 Background

Q1. How many years have you worked at Tulip Med and at a clinic in Russia, and what is your specialty?

Physician 1. I specialise in dermatology and aesthetic medicine. I worked at a private clinic in Yekaterinburg for four years and have been at Tulip Med for two years.

Physician 2. I am an aesthetic medicine physician. I worked at a clinic in Novosibirsk for five years and have been at Tulip Med for about a year. My work mainly involves consultations about aesthetic procedures and follow-up care.

Q2. How did you come to work at each clinic?

Physician 1. A colleague I had worked with in dermatology invited me to join the clinic in Yekaterinburg. I met the Tulip Med team at a professional event and later received an offer to join the clinic.

Physician 2. I applied for an opening in Novosibirsk when the clinic was expanding its team. After moving to Astana, I applied to Tulip Med. At the interview, we talked a lot about how the clinic follows up with patients after a visit.

2 Similarities and differences

Q3. What stayed the same in your approach to patients at both clinics?

Physician 1. I started by asking what the patient wanted to achieve. Then I explained which options we could consider and what expectations were realistic. I followed that sequence at both clinics.

Physician 2. At both clinics, I made time for questions and discussed how to reach us after the appointment. I want people to leave with a clear plan, even if all we did that day was talk.

Q4. What did you have to change when moving from one country to the other, for example in communication, patient expectations, documentation or procedure selection?

Physician 1. In Astana, I had to get used to how the front desk passed information to the physician and how the team documented consultations. I also found it useful to ask early on whether the patient wanted my opinion or was expecting to see a particular colleague.

Physician 2. I mainly changed how I explained the schedule. At Tulip Med, I more often saw people with limited time in the city. When discussing options, I had to establish early whether they could return for a check-up. I learned the clinic's forms and booking process on the job.

Q5. How do patient expectations differ in Kazakhstan and Russia?

Physician 1. At my clinic in Yekaterinburg, patients more often asked whether I would personally oversee the full course of care. At Tulip Med, some came on the recommendation of the clinic as a whole and were happy to choose a physician with the front desk. That is an observation about two clinics.

Physician 2. I saw a lot of similarities. In both Novosibirsk and Astana, new patients wanted to understand costs and timing in advance. The differences I noticed had more to do with their previous experience of procedures and how far away they lived.

3 Trust and the founder

Q6. How do patients choose a clinic, for example by physician, brand, recommendation or price?

Physician 1. A recommendation often leads a person to a specific name. They then look up where the physician practises and consider the price. When referred by someone they know, they sometimes arrive with fairly clear expectations of the consultation.

Physician 2. I more often see patients compare several options. They read reviews, ask about costs and see how clearly the clinic answers before booking. Between two similar options, they may choose the clinic that better explained what the appointment includes.

Q7. How much does the name of the physician or founder matter to patients? How did this compare at Tulip Med and the Russian clinic?

Physician 1. In Yekaterinburg, the names of individual physicians carried more weight. At Tulip Med, Karlygash's name sometimes gives people their first introduction to the clinic. Some are happy to see her colleagues once they understand each specialist's role.

Physician 2. In Novosibirsk, patients rarely asked about the founder, but they read reviews of the physician they planned to see. At Tulip Med, Karlygash can help spark interest. Before booking, I would still introduce the patient to the physician who will see them.

Q8. How often does a physician decline to perform a procedure or advise waiting? How do patients respond?

Physician 1. It comes up regularly, although I have not counted how often. Some patients are surprised, especially if they have already decided what they want. When you explain why you suggest waiting and what can be discussed later, many accept it calmly.

Physician 2. In my practice it happens from time to time. Some patients appreciate the explanation and others are disappointed, especially if they made a trip for it. That is why it helps to explain at booking that the consultation may end with advice to postpone the procedure.

4 Patients from abroad

Q9. Have patients from Russia or other countries visited Tulip Med? What was their main barrier, for example price, travel, timing, language or trust?

Physician 1. I saw a few patients from Russia who were already planning to visit Astana for family reasons. They mostly asked whether the physician they wanted was available on their travel dates. Those cases tell us little about people who would make a trip specifically for care.

Physician 2. I dealt with enquiries from abroad, but I did not always know whether the visit took place. The question that came up most often was whether a second trip would be needed. For those people, uncertainty about timing could matter more than the consultation fee.

Q10. What matters most to a patient travelling from far away before arrival, during the visit and after returning home?

Physician 1. Before travelling, patients need to know who will see them and how much time to allow. During the consultation, they need to understand their options. Back home, it helps to have a written summary and know who will answer their questions.

Physician 2. I would explain in advance what could be addressed in a single visit and what that depends on. At the appointment, we would refine the plan around the departure date. Back home, the patient should have a clear way to contact the clinic and a plan for an in-person check locally if needed.

Q11. How would you organise follow-up for a patient who lives in another country?

Physician 1. Before departure, I would agree with the patient on when to check in, who would be responsible and how to contact them. The written plan should state which questions can be addressed remotely and when an in-person assessment is needed.

Physician 2. Before booking, I would establish whether the patient could access an in-person assessment at home if needed. If follow-up is difficult to arrange, that will affect what type of visit we can offer.

5 Pricing and channels

Q12. How do patients compare the total cost of travelling with the price of receiving care at home?

Physician 1. People already travelling to Astana usually consider the extra cost of the appointment and the time it takes. Someone planning a separate trip for a particular physician or a second opinion weighs whether the consultation is worth the journey.

Physician 2. I would compare the two scenarios in full. Travelling means transport, accommodation, time away from work and a possible return visit. The local option includes the consultation and the same follow-up stages if needed. That makes it easier to see where the difference comes from.

Q13. How do patients usually hear about the clinic, for example through recommendations, social media, search or messaging apps? Did you notice differences between Kazakhstan and Russia?

Physician 1. In Yekaterinburg, I more often heard, “Someone recommended you.” At Tulip Med, some patients mentioned first seeing Karlygash explain something on social media and then messaging the clinic. These are different ways of discovering a clinic, so I would ask about the first source separately at booking.

Physician 2. In Novosibirsk, patients often said they found the clinic through search and then read reviews. In Astana, I also hear about search and recommendations. A messaging app is often how people book after hearing about the clinic elsewhere.

6 Assessment and recommendations

Q14. Do you think Tulip Med should attract patients from Russia to Astana, and what would matter most?

Physician 1. I would try starting with consultations for people seeking a second opinion. Karlygash could explain her approach to those consultations, and the clinic could see how many people are then willing to pick an appointment date.

Physician 2. I would first look at a few incoming enquiries and see whether we can offer patients a workable plan for their visit and follow-up. If the trip proves too complicated even for interested patients, a broad campaign is unlikely to solve that problem.

Q15. What would you advise the clinic to avoid?

Physician 1. I would avoid advertising that could leave people expecting an appointment with Karlygash herself. It is better to introduce the team early and explain who the patient will see and why.

Physician 2. I would avoid discounts that encourage people to buy a ticket before they understand the terms of the visit and whether a follow-up assessment is feasible. I would also avoid judging the campaign solely by the number of messages.

Appendix B. Survey of five adults from Russia

Conducted on Instagram, 3–10 August 2026. Thirty users invited, five responded. Answers translated from Russian by the author.

Questions: (1) Would you consider travelling to Astana for a consultation with a cosmetologist or aesthetic medicine physician? (2) What would be the main barrier? (3) Would you go for one visit, or would you need the option of a repeat visit? (4) Where would you look for such a clinic? (5) Does it matter that the well-known founder sees you, or is any physician of the clinic fine? (6) What should the clinic explain before booking for you to decide?

Anna (29, Omsk)

1. Yes. I would consider a trip for a second opinion if I found a specialist with experience in my issue.

2. Trust. I want to understand who exactly will consult me and how experienced the physician is.

3. I would go for one consultation. If a repeat in-person visit is needed, it is important for me to know in advance.

4. First I would ask acquaintances, then look at the clinic’s website and reviews of the physician.

5. The founder’s name would attract attention. If another physician has suitable experience, I will happily book with them.

6. Who conducts the appointment, how much it costs and what I get at the end of the consultation, for example a written summary.

Maxim (38, Yekaterinburg)

1. No. For an ordinary consultation I would look for a physician closer to home.

2. Time. The trip may take more time than I am willing to give to a single appointment.

3. I would not go just for one visit. If I were already in Astana for work, I could book.

4. Through search and maps. Then I would check reviews and the schedule.

5. The founder’s name is secondary for me. I would look at the qualifications of the physician who will see me.

6. How long the appointment takes, and whether I can book a specific day without an additional visit.

Elena (44, Novosibirsk)

1. Possibly. If the appointment could be combined with a trip to relatives in Astana.

2. Follow-up after the visit. I want to know whom to contact when I get home.

3. One visit is enough if the consultation ends there. If follow-up is needed, I want to understand in advance whether an in-person check is possible near home.

4. I would ask relatives in Astana, then write to the clinic by messenger.

5. A physician from the team is enough for me, if the clinic explains why they are suitable and who will be in touch after the appointment.

6. Whether a repeat check will be needed, who will answer questions after I return, and how to get recommendations in writing.

Daria (25, Tyumen)

1. Possibly. I would first compare what the trip would cost with the price of a similar consultation at home.

2. Price. I would need to add up the road and accommodation together with the appointment.

3. I would consider one visit. If I need to go again, I would rather choose a physician in Tyumen.

4. I would start with social media, then check the physician’s information and reviews through search.

5. The founder’s name may spark interest. For booking, it matters more to me to understand which physician deals with my issue.

6. What the price of the consultation includes, what may cost extra, and what the cancellation terms are.

Irina (52, Chelyabinsk)

1. Yes, if I have long followed a particular physician and want to hear specifically their opinion.

2. The journey. I would decide by how convenient it is to get there and whether I can get an appointment on suitable dates.

3. I would go for one consultation if it is confirmed that the right physician will see me. I would plan a repeat trip separately.

4. Through recommendations and professional videos. I would book through the clinic’s official website.

5. In my case it matters that the founder herself sees me, if I am travelling specially for her opinion.

6. Who exactly will see me, what happens if the physician is replaced, and whether I can reschedule if my travel plans change.